

February 20, 2026

Dr. Thomas Keane
Assistant Secretary for Technology Policy/Office of the National Coordinator for Health
Information Technology
Department of Health and Human Services
Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

Submitted electronically via www.regulations.gov

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Assistant Secretary Keane:

On behalf of the Heart Failure Society of America (HFSA), we appreciate the opportunity to provide input in response to the request for information (RFI) on how the Department of Health and Human Services (HHS) can help shape the development, evaluation, and deployment of artificial intelligence (AI) in clinical care. HFSA is a multidisciplinary organization working to improve and expand heart failure care through collaboration, education, research, innovation, and advocacy. HFSA members include physicians, scientists, nurses, nurse practitioners, pharmacists, and patients.

Through this RFI, HHS seeks feedback on how regulations, reimbursement policies, and research and development efforts can be shaped to better support the rapid adoption and effective use of AI in clinical care, while fostering public trust and aligning with federal priorities to enhance productivity, reduce burden, lower health care costs, and improve health outcomes.

Heart failure offers a compelling use case for the responsible adoption of AI in clinical care. It is a common, resource-intensive chronic condition that is managed across complex, longitudinal care pathways and workflows. Approximately 6.7 million Americans over the age of 20 are currently living with heart failure and the prevalence is expected to increase substantially over the coming decades, rising to 11.4 million by 2050. The lifetime risk of developing heart failure is 24%, meaning approximately one in four individuals will develop the condition. In 2022, heart failure was a contributing cause in more than 425,000 deaths, accounting for 45% of all cardiovascular deaths nationwide.¹

Significant gaps, variation, and disparities persist in the use of guideline-recommended therapies for heart failure. Fewer than one in four eligible patients with heart failure with reduced ejection fraction receive comprehensive quadruple guideline-directed medical therapy (GDMT), despite clear evidence of its benefit. Broader and more consistent implementation of these therapies could have a profound impact on outcomes, with estimates suggesting that optimal uptake could prevent approximately 1.19 million deaths globally each year.²

This creates clear opportunities for AI to improve operational efficiency, reduce clinician burden, and increase reliability of delivering evidence-based care. This potential is reflected in a recent research and development initiative offered through HHS' Advanced Research Projects Agency for Health (ARPA-H), which seeks to advance agentic AI technologies as "clinician extenders." Through

¹ Fonarow GC, et al. HF STATS 2025: Heart Failure Epidemiology and Outcomes Statistics—An Updated 2025 Report From the Heart Failure Society of America. *Journal of Cardiac Failure*. 2025;32(2):439-498.

² Fonarow GC, et al. HF STATS 2025.



the Agentic AI-Enabled Cardiovascular Care Transformation (ADVOCATE) program, ARPA-H is soliciting proposals to develop agentic AI systems to support clinical care for patients with heart failure and individuals recovering from myocardial infarction.

At the same time, heart failure care is inherently high risk. Poorly governed or inadequately evaluated AI tools could cause patient harm through preventable hospitalizations, medication-related adverse events, or delayed escalation of care. Accordingly, the adoption of AI in clinical care must be accompanied by robust governance, rigorous prospective evaluation, and continuous post-deployment monitoring to ensure safety, effectiveness, and equity across patient populations.

Recommended Principles

To ensure that AI is used appropriately in clinical care, HFSA recommends that HHS consider the following principles when establishing policies to support the deployment of AI technologies:

- **Establish risk-proportionate clinician governance and accountability for AI used in clinical care.** AI tools that inform diagnoses, triage, medication titration, escalation decisions, or discharge planning should be deployed only with clear clinical ownership and defined human oversight. HHS should promote governance expectations that are proportionate to a tool's clinical impact, including an explicit intended-use statement, documented workflows for clinician override, and auditable logs of AI outputs and clinician actions. These measures clarify accountability, preserve appropriate clinical judgment, and support public trust while enabling innovation.
- **AI-enabled care should be designed to address the full heart failure continuum.** AI initiatives in heart failure should be grounded in an understanding of heart failure as a continuum. Approximately one-third of U.S. adults are at risk for heart failure (Stage A) and nearly 24-34% have pre-heart failure (Stage B). While prevalence of stage C2D and Stage D heart failure requires further examination, it has been estimated that 5-7% of patients with symptomatic heart failure have advanced heart failure. Rising rates of obesity diabetes, hypertension, and chronic kidney disease are accelerating progression across this continuum.³ Despite clear evidence-based therapies, substantial gaps, variations, and disparities in use of guideline-recommended therapies persist, as noted above. Effective AI must support surveillance, prevention, early detection, treatment optimization, and advanced care across all stages of heart failure.
- **Align reimbursement and federal incentives to support implementation, monitoring, and value-based adoption of AI-enabled care.** Many clinically meaningful AI tools require substantial upfront investment in integration, workflow redesign, training, and ongoing surveillance. These costs are often difficult for providers to absorb without a reliable reimbursement pathway. HHS, largely through the Centers for Medicare and Medicaid Services (CMS), should prioritize payment approaches that reward demonstrated clinical utility and burden reduction, while avoiding per-alert or per-use incentives that may drive volume without value. Time-limited implementation support and demonstration models for AI-enabled care pathways would reduce adoption friction while promoting responsible scale. CMS should look to offer incentives to providers that reward safe deployment of AI.
- **Shift evaluation expectations from retrospective accuracy to prospective clinical utility, safety, and equity in real workflows.** HHS should invest in research evaluating whether and how AI improves outcomes and reduces clinician burden without introducing new harms in the real world. Retrospective performance metrics alone are insufficient to justify clinical deployment. Evaluation frameworks should incorporate human factors

³ Fonarow GC, et al. HF STATS 2025.

testing, alert burden and usability metrics, calibration and subgroup performance, and clinically meaningful safety endpoints. HHS can accelerate responsible adoption by supporting pragmatic, multi-site randomized trials and post-deployment monitoring that measure real-world impact.

- **Establish comprehensive lifecycle evaluation framework for AI tools.** Appropriate HHS authorities should oversee pre-deployment evaluation and lifecycle regulation of AI tools that meet criteria for Software as a Medical Device, including requirements for clinical validation, transparency, and post-market performance monitoring to address model drift and safety risks in heart failure populations. In particular, the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC) should establish interoperability and auditability standards to ensure AI tools integrate reliably with electronic health records and support clinician interpretability and governance. CMS should also align reimbursement and quality measurement approaches with broader CMS initiatives, such as the Ambulatory Specialty Model and the Advancing Chronic Care with Effective, Scalable Solutions Model, to support coordinated adoption of high-value AI-enabled care.

HFSA appreciates the opportunity to provide input as HHS continues to evaluate policies to responsibly accelerate adoption of AI in clinical care. We encourage HHS to maintain ongoing engagement with representative clinical experts and patient populations in the development of AI-related policies to ensure that resulting recommendations are accurate, precise, and grounded in real-world clinical practice. If you have questions or require additional information, please contact Sue Ramthun at sramthun@hfsa.org.

Sincerely,



Kenneth B. Margulies, MD
President, Heart Failure Society of America